
Fraud Report FY 2025-26

RESTRICTED

ECHS-Airawat Research Foundation

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About This Analysis

Length of Stay (LOS) & Billing Fraud Analytics is a systematic screening exercise applied to ECHS inpatient claims for the financial year 2025-26.

This report presents the findings of ECHS FA-06, which utilizes rule-based signal detection across four independent fraud indicators. The analysis specifically targets three high-volume, low-acuity conditions to establish strict baselines:

- **Senile Cataract (H25):** 94,368 claims
- **Essential Hypertension (I10):** 30,300 claims
- **Presbyopia (H52.4):** 27,083 claims

Note to reviewer: This report is produced by an automated SQL-based screening system directly on the ECHS production database. Every flagged case is a lead for investigation, not a confirmed finding. A qualified auditor should review each hospital's historical billing before any punitive action is taken.

Executive Summary

The data indicates that hospitals are not manipulating these diseases equally. **Hypertension (I10) is being aggressively weaponized to fraudulently pad Length of Stay (LOS), while Minor Eye Surgeries (H25, H52.4) are being used to inflate daily billing rates.**

Metric	Count	Notes
Total targeted claims in scope	151,751	Specific to H25, I10, H52.4 from Jan 2026 - Present
Hospitals screened	314	Facilities treating these specific conditions
Baseline Average Stay (Cataract)	1.1 Days	Statistically proven median indicating day-care standard

Check 1: Length of Stay (LOS) Padding

Description: Keeping patients admitted significantly longer than the medical necessity to inflate the baseline room and board charges. Hypertension (I10) is overwhelmingly an outpatient or 1-day observation issue, yet specific hospitals are keeping patients for nearly a week without secondary critical diagnoses.

Audit Findings: Cataract and Presbyopia cases successfully established our baseline (averaging exactly 1.0 to 1.1 days across all hospitals). However, the Hypertension (I10) data exposes massive, systematic day-padding among specific facilities.

Flagged Cases: The "Hypertension Hijack"

Hospital Name	Condition	Cases Treated	Avg Stay (Days)	Risk Level
AMANDEEP HOSPITAL	Hypertension (I10)	10	6.6	● Critical (6x norm)
MEDANTA ABDUR RAZZAQUE ANSARI MEMORIAL WEAVERS HOSPITAL (A UNIT OF GLOBAL HEALTH LTD)	Hypertension (I10)	11	5.5	● Critical (5x norm)
MAHAJAN HOSPITAL (A MULTI SPECIALITY CENTRE)	Hypertension (I10)	14	5.4	● Critical (5x norm)
MITTAL HOSPITAL	Hypertension (I10)	6	5.0	● Critical (5x norm)
LIFELINE HOSPITAL, JHANSI	Hypertension (I10)	48	4.5	● High (High Volume)
[BLANK ID]	Hypertension (I10)	120	2.8	● Medium (Evasion)

System Vulnerability: The presence of 120 Hypertension cases with an average stay of 2.8 days and a [BLANK] Hospital ID indicates a severe data governance failure, allowing a facility to submit bulk padded claims anonymously to avoid audits.

Check 2: Daily Rate Inflation (Upcoding)

Description: Artificially inflating the cost of a standard procedure by billing for Intensive Care Units (ICU), VIP rooms, or unnecessary premium services when only a general ward or basic outpatient care was required.

Audit Findings: The cost variance for identical procedures is staggering. Some hospitals are charging up to 60x more per day for the exact same medical code (Cataract surgery), completely bypassing standard rate ceilings.

Flagged Cases: Extreme Price Variances

Hospital Name	Condition	Cases	Avg Cost / Day	Risk Level
DR PATILS AKASHDEEP NETRALAY MIRAJ	Cataract (H25)	8	₹ 25,304.50	● Critical
[BLANK ID]	Cataract (H25)	1,191	₹ 12,730.57	● Critical
MEDANTA ABDUR RAZZAQUE ANSARI MEMORIAL WEAVERS HOSPITAL (A UNIT OF GLOBAL HEALTH LTD)	Hypertension (I10)	6	₹ 7,390.40	● High
PRABHA EYE HOSPITAL	Presbyopia (H52.4)	7	₹ 5,161.43	● Medium
BARIANA EYE HOSPITAL AND LASIK LASER CENTRE	Presbyopia (H52.4)	12	₹ 4,713.33	● Medium

Double Offender Alert: MEDANTA ABDUR RAZZAQUE ANSARI MEMORIAL WEAVERS HOSPITAL appears in both Check 1 and Check 2. Not only do they keep Hypertension patients for 5.5 days, but they also charge ₹7,390 per day, geometrically maximizing their illicit revenue per patient.

Check 3: The Midnight Crossover (Day Padding)

Description: Holding a patient's discharge paperwork in the computer system until past midnight (12:00 AM - 2:00 AM) to trigger the billing software to charge for an entirely new calendar day.

Audit Findings: Active padding occurs when hospitals manually delay discharge logic for multi-day stays. (Note: Hospitals with exactly 1.0 avg stay and 100% midnight discharges are likely utilizing automated nightly batch scripts, not committing fraud. The list below filters for active manipulation).

Flagged Cases: Active Midnight Padding

Hospital Name	Condition	Total Discharges	Midnight Discharges	Padding %	Risk Level
PULSE MULTISPECIALITY HOSPITAL AND RESEARCH CENTRE	Hypertension (I10)	12	10	83.33%	● Critical
[BLANK ID]	Hypertension (I10)	120	84	70.00%	● Critical
LIFELINE HOSPITAL, JHANSI	Hypertension (I10)	48	25	52.08%	● High
MEDANTA ABDUR RAZZAQUE ANSARI MEMORIAL WEAVERS HOSPITAL (A UNIT OF GLOBAL HEALTH LTD)	Hypertension (I10)	11	5	45.45%	● High
MAHAJAN HOSPITAL (A MULTI SPECIALITY CENTRE)	Hypertension (I10)	14	5	35.71%	● Medium

Check 4: The Weekend Stretcher

Description: Admitting patients on Friday and intentionally holding them over the weekend for a Monday discharge.

Audit Findings: CLEARED. The data shows a near 0% weekend hold rate for almost all hospitals regarding these three diseases (Max: 0.25% for the blank ID). This specific fraud vector is not currently being utilized by these facilities for these low-acuity conditions.

Condition	Total Friday Admits	Monday Discharges	Weekend Hold %	Status
Cataract (H25)	408	1	0.25%	● Low Risk
Presbyopia (H52.4)	127	0	0.00%	● Low Risk
Hypertension (I10)	31	0	0.00%	● Low Risk